PRACTICE EXAM 18 / EXAM SET 03

Domain 4: Patient Care Coordination and Education

12 scored questions	4 answer choices per question	Answer key + explanations included

Instructions

Choose the single best answer (A, B, C, or D) for each question. Mark your responses before checking the answer key and detailed explanations at the end of this document. These practice questions are not official NHA examination items.

QUESTIONS

Q01 | Preventive Care and Care Gaps

SCENARIO During previsit planning, an assistant finds two appointments at different facilities for the same screening colonoscopy. Both were booked from the same provider recommendation after separate outreach calls. The patient believes both appointments are necessary. Clinic policy requires the ordering team to resolve possible duplicate services before staff cancel a booking.

Which action best prevents unnecessary duplication while preserving the screening plan?

A. Keep both appointments and tell the patient that two facilities provide a more complete screening.

B. Cancel the later appointment without checking which facility the ordering team intended to use.

C. Ask each facility to proceed unless the other facility sends a completed procedure report.

D. Flag the duplicate bookings for the ordering team and coordinate the confirmed single screening arrangement.

Q02 | Referrals and Care Transitions

SCENARIO A young adult with a developmental disability is transferring from a pediatric specialty clinic to an accepting adult clinic. The patient has authorized caregiver participation. The current care plan documents use of a communication board and assistance with organizing appointments, but the draft transition summary contains only diagnoses and medications.

Which addition would most improve the usefulness of this transition handoff?

A. The documented communication and appointment-support needs, along with the authorized support contact.

B. A statement that the adult clinic should obtain practical support information at a later visit.

C. The complete pediatric billing history in place of information about functional support needs.

D. An assumption that the caregiver will arrange services without the adult clinic needing the care plan.

Q03 | Patient Education and Self-Management Support

SCENARIO A child's prescription label specifies 3 mL of an oral liquid per dose. The pharmacist has supplied a 5 mL oral syringe with markings every 0.2 mL. During reinforcement of the pharmacist's instructions, the caregiver says they usually measure liquids with a kitchen teaspoon. The assistant is trained and authorized to reinforce the supplied measuring instructions.

Which teaching approach best supports accurate measurement of the prescribed volume?

A. Show how to estimate part of a kitchen teaspoon and ask whether the estimate seems reasonable.

B. Use the supplied oral syringe to demonstrate 3 mL, then have the caregiver show the same volume.

C. Use a 30 mL medicine cup marked every 5 mL and estimate a level between its markings.

D. Show how to fill the 5 mL syringe completely so the caregiver can use a clearly marked amount.

Q04 | Community Resources and Access Barriers

SCENARIO A patient wants to attend provider-recommended counseling but cannot miss daytime shifts without losing wages needed for rent. The clinic resource directory lists several counseling programs, including one with evening sessions. Its current intake availability has not been confirmed, and the patient agrees to assistance.

Which action most directly addresses this access barrier?

A. Book a daytime session and send an additional reminder so the patient can plan around it.

B. Recommend postponing counseling until the patient can change employment.

C. Verify evening intake availability and help connect the patient with a program that fits the work schedule.

D. Provide a general counseling brochure and record the appointment barrier as resolved.

Q05 | Referrals and Care Transitions

SCENARIO A provider orders an outpatient imaging study for a patient who cannot transfer independently from a wheelchair. The patient's documented plan requires trained staff and a mechanical transfer lift. The proposed imaging site confirms it has no lift. Another site may have the necessary equipment, but this has not been verified.

Which coordination action best prepares for a safe, workable referral?

A. Tell the patient's family to lift the patient at the original site so the appointment can proceed.

B. Verify an appropriate receiving site with the required transfer resources and coordinate it through the ordering team.

C. Book the original site and assume its staff will obtain suitable equipment on the day of the study.

D. Cancel the imaging order because the originally proposed facility cannot meet the transfer need.

Q06 | Patient Education and Self-Management Support

SCENARIO A patient had been forgetting several scheduled doses of a daily medication each week. The prescribing team kept the regimen unchanged, and the assistant helped the patient set reminders for the prescribed times and use a dose checklist. At follow-up, the assistant evaluates whether this self-management teaching has helped.

Which observation provides the strongest evidence that the reminder strategy is helping?

A. The patient reports that the phone reminders are reassuring and is confident the medication will work better.

B. The patient demonstrates how to find and edit the phone reminders without referring to the medication schedule.

C. The patient brings blank dose checklists for the coming month and reports that they are easy to read.

D. The patient explains the schedule correctly and reviews a seven-day checklist showing doses taken at prescribed times.

Q07 | Community Resources and Access Barriers

SCENARIO An older adult reports difficulty stepping into the bathtub and completing bathing tasks. There is no reported recent injury or new acute symptom. The provider asks the assistant to identify a community clinical service that can evaluate daily activities and home adaptations, then return the resource information for the provider's referral decision.

Which resource best matches the requested service?

A. A home-based occupational therapy service that evaluates daily activities and environmental adaptations.

B. A home cleaning service that performs scheduled household cleaning tasks.

C. A meal delivery service that brings prepared lunches to the home.

D. A social telephone program that provides conversation and regular check-in calls.

Q08 | Preventive Care and Care Gaps

SCENARIO A provider orders a recommended vaccine, but the clinic has no remaining stock. The provider authorizes the same ordered vaccine at a partner vaccination site. The patient agrees, and the partner can confirm product availability and send an administration record afterward. The vaccine has not yet been given.

Which action best supports completion of this preventive service?

A. Mark the vaccine administered when the offsite appointment is booked.

B. Select a different vaccine from clinic stock so the patient can receive an injection today.

C. Coordinate the authorized partner visit and track return of the administration record after vaccination.

D. Close the vaccine task because the clinic cannot provide the ordered product today.

Q09 | Patient Education and Self-Management Support

SCENARIO A patient following a dietitian's nutrition plan asks how to choose reliable online background information. The assistant is reviewing descriptions of four educational pages. The purpose is to support the existing plan, not to select a new diet.

Which description offers the strongest evidence of a trustworthy educational source?

A. A public health agency page identifies its expert review process, review date, and supporting references.
B. A frequently updated supplement store page ranks diets using testimonials from its customers.
C. A popular personal blog lists a large follower count but provides no author qualifications or references.
D. A reposted article displays a medical logo but does not identify its original author, date, or source.

Q10 | Community Resources and Access Barriers

SCENARIO A patient requesting help with temporary housing was referred to a community program that serves only a neighboring county. The program has confirmed that the patient is outside its service area. The patient still needs assistance and agrees to contact with the clinic's housing resource navigator. No new program has been identified.

Which follow-up best addresses the failed resource connection?

A. Send the same referral again and advise the patient that another attempt should establish eligibility.
B. Document that housing assistance was provided because a resource name was given.
C. Promise that the clinic's navigator can secure housing before reviewing any local options.
D. Update the navigator about the service-area denial and seek a verified resource serving the patient's location.

Q11 | Preventive Care and Care Gaps

SCENARIO After discussing an individual patient's circumstances, the provider documents a decision to stop a particular screening and records the patient's agreement. The automated preventive-care list continues to send generic reminders for that screening. Clinic policy allows designated staff to update outreach exceptions from documented provider decisions.

How should the assistant address the continuing reminders?

A. Schedule the screening because an automated reminder overrides an individualized decision.
B. Submit the documented decision through the outreach-exception process and preserve the clinical record.
C. Enter a completed screening result even though no test was performed.
D. Delete the provider's decision so the clinical record matches the default reminder list.

Q12 | Referrals and Care Transitions

> **SCENARIO** A discharge planner confirms that a patient is going to a rehabilitation facility rather than returning home. The primary care coordination file still lists the original home destination and a planned home nursing start. The designated nurse coordinator directs service-plan changes; the assistant may send authorized transition information to the receiving care team.

Which action best supports an accurate handoff after this destination change?

A. Keep the home destination in the coordination file until the patient eventually returns home.

B. Cancel the home service order independently and assume the rehabilitation facility will obtain all records.

C. Update the nurse coordinator with the confirmed destination and send the authorized handoff to the receiving rehabilitation team.

D. Send the original home plan only to the home nursing agency and mark the transition complete.

ANSWER KEY & EXPLANATIONS

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01 D	02 A	03 B	04 C	05 B	06 D
07 A	08 C	09 A	10 D	11 B	12 C

DETAILED EXPLANATIONS

Q01 | Correct answer: D

Flag the duplicate bookings for the ordering team and coordinate the confirmed single screening arrangement.

Two bookings from one recommendation create a coordination discrepancy rather than a need for two procedures. The assistant should obtain the ordering team's direction, then coordinate one confirmed arrangement and the necessary booking updates instead of independently choosing a facility or leaving both active.

Q02 | Correct answer: A

The documented communication and appointment-support needs, along with the authorized support contact.

The receiving team needs practical information about established supports to make the new care setting usable for this patient. Diagnoses and medication lists alone do not describe how to communicate or coordinate appointments effectively; the assistant should transmit documented needs rather than create a new plan.

Q03 | Correct answer: B

Use the supplied oral syringe to demonstrate 3 mL, then have the caregiver show the same volume.

The supplied oral syringe can measure the prescribed 3 mL at an actual graduation, and observing the caregiver measure that volume checks the teaching. Kitchen spoons vary in volume, and estimating on a coarsely marked cup or filling to the syringe's capacity would not reliably measure the prescribed dose.

Q04 | Correct answer: C

Verify evening intake availability and help connect the patient with a program that fits the work schedule.

The stated barrier is the conflict between service hours and essential work, so checking an evening program targets that specific obstacle. More reminders or a brochure do not change service availability, and the assistant should not assume the patient must change jobs to obtain care.

Q05 | Correct answer: B

Verify an appropriate receiving site with the required transfer resources and coordinate it through the ordering team.

The receiving service must be able to meet the transfer requirements already documented in the care plan. Verifying those capabilities before booking avoids relying on an unsafe improvised transfer; the assistant facilitates an appropriate site without canceling the clinical need for imaging.

Q06 | Correct answer: D

The patient explains the schedule correctly and reviews a seven-day checklist showing doses taken at prescribed times.

Correct explanation of the schedule together with a record of following it provides evidence that the strategy is being applied. Setting up or liking a reminder does not demonstrate use of the medication plan; the checklist is patient-reported follow-up evidence rather than independent proof that every dose was swallowed.

Q07 | Correct answer: A

A home-based occupational therapy service that evaluates daily activities and environmental adaptations.

Occupational therapy addresses performance of daily activities and can evaluate how the home environment affects those activities. Cleaning, meal delivery, and social calls may meet other needs but do not provide the clinical evaluation requested; the assistant supplies resource information for the provider's decision.

Q08 | Correct answer: C

Coordinate the authorized partner visit and track return of the administration record after vaccination.

The provider has authorized a specific alternative location, so the assistant can coordinate delivery of the existing preventive plan and verify actual administration. A booking is not an administration record, and lack of local stock does not authorize changing the vaccine product or abandoning follow-up.

Q09 | Correct answer: A

A public health agency page identifies its expert review process, review date, and supporting references.

Transparent authorship or institutional responsibility, expert review, current review information, and supporting references help establish educational reliability. Popularity, frequent posting, testimonials, and a medical-looking logo do not establish the quality of the underlying information; online background material does not replace the individualized nutrition plan.

Q10 | Correct answer: D

Update the navigator about the service-area denial and seek a verified resource serving the patient's location.

The first resource cannot serve this patient, so useful follow-up must account for the confirmed geographic restriction. An updated search and specific connection are needed; repeating an ineligible referral or recording it as successful does not resolve the barrier, and placement cannot be guaranteed.

Q11 | Correct answer: B

Submit the documented decision through the outreach-exception process and preserve the clinical record.

The reminder system should reflect the documented individualized prevention plan through the approved exception process. Stopping screening after a documented decision is different from completing a test; recording a false result or discarding the decision would make the record inaccurate.

Q12 | Correct answer: C

Update the nurse coordinator with the confirmed destination and send the authorized handoff to the receiving rehabilitation team.

The actual receiving setting must be reflected in coordination and receive the authorized information needed for continuity. Notifying the designated nurse allows service-plan changes to be directed appropriately; the assistant should not cancel clinical services independently or assume records will arrive without a handoff.

End of Practice Exam